

Final Procedure Concept Library

Inpatient DRG Payment Integrity | 34 concepts, one numbering scheme (P01–P34), six categories | Plain-language rationale, guideline anchor, and run-status for every concept | SQL companion: FINAL_concept_library.sql

This replaces every earlier concept list

Everything built across the previous documents has been re-verified, de-duplicated and renumbered into a single list. Old IDs (C01–C50, and the A/B/C/D/E/F/G red-flag checks) are retired — a crosswalk is on the last page so nothing you already started is lost. Each pattern in this library was tested against a code that should match and a near-miss code that should not, and all 26 pattern families passed. Where I am not fully confident of a body-part or device value, the concept is written deliberately wide and marked **VERIFY** rather than guessed at.

The October problem, and how it is handled

It is August 2026. FY2027 begins 1 October 2026, so you have **no claims** with the new DRGs (523–525, 449, 400, 403, 404, 210, 211, 731–733) and none billed under grouper version 44. Three rules follow, and they are applied throughout this document:

Situation	Concepts affected	What this library does
Concept has no date logic at all	P01–P30 (28 of 34)	Runs today, unchanged. No date filter appears anywhere in the SQL.
Concept targets a NEW FY2027 DRG	P31 (FY2027 family gates)	Runs today in BACKTEST mode against the predecessor DRGs the new families were carved out of. Hit rates transfer; only the payment weights change in October.
Concept IS a date rule	P33 (deleted-DRG edit)	Cannot run before October — the check is 'was this DRG valid on the discharge date?'. Parked as a day-one October job. A readiness query is provided instead, sizing how many of your claims sit in the DRGs about to be deleted.

Bottom line: 33 of 34 concepts run on the claims you have today. Only the deleted-DRG edit waits for October.

How to read a concept, and what the two labels mean

NO CHART NEEDED means the claim contradicts itself — you can prove the finding from claims data alone. These are the cheapest to run, the fastest to confirm, and the hardest for a provider to appeal, because you are not debating clinical judgment. Start here. **CHART NEEDED** means the claims data only tells you which claims are worth pulling; a reviewer then reads the operative note to confirm. These are worth more per claim but cost review time. Every concept also carries its **guideline** — the written rule that makes the finding stick. A finding without a guideline is an opinion, and opinions lose appeals.

Summary — all 34 concepts at a glance

ID	Concept	Chart?	Runs today?
CATEGORY 1 — DATA & DATE INTEGRITY (run these first)			
P01	Procedure dated outside the hospital stay	No	Yes
P02	Procedure dates impossibly far apart on one claim	No	Yes
P03	Same code, same date, billed twice	No	Yes
CATEGORY 2 — IMPOSSIBLE ON ITS FACE (highest confidence)			
P04	Over 96 hours of ventilation in a stay shorter than that	No	Yes
P05	Two conflicting ventilation duration codes	No	Yes
P06	Two separate 'over 96 hour' ventilation episodes	No	Yes
P07	Top-paying DRG 003/004 with no tracheostomy code	No	Yes
P08	A throat tube or tube swap treated as a tracheostomy	No	Yes
CATEGORY 3 — PROCEDURE CONTRADICTS THE DIAGNOSIS			
P09	Fracture 'put back in place' that was never displaced	No	Yes
P10	Deep debridement with no deep-infection diagnosis	Yes	Yes
P11	Spine bone surgery with a disc diagnosis (or reverse)	Yes	Yes
P12	Brain artery coded when the neck artery was treated	Yes	Yes
P13	Spinal cord release coded when a nerve root was freed	Yes	Yes
P14	Patient's own skin graft with no donor site	No	Yes
CATEGORY 4 — CODED TWICE / UNBUNDLED			
P15	Bleeding control plus the operation that stopped it	Yes	Yes
P16	A 'look inside' billed with the surgery it was part of	Yes	Yes
P17	Keyhole and open version of the same operation	Yes	Yes
P18	Aborted operation billed as if completed	Yes	Yes
P19	Whole-organ removal and partial removal together	Yes	Yes
P20	Spinal cage plus a separate bone-graft code, same level	Yes	Yes
P21	Joint liner swap billed as a full revision	Yes	Yes
CATEGORY 5 — THE NOTE MUST BE READ			
P22	Wound 'cut out' when the note says only washed	Yes	Yes
P23	Airway biopsy or lavage billed as lung surgery	Yes	Yes
P24	Surgical claim with no operative report	Yes	Yes
P25	Single-level fusion billed as multi-level	Yes	Yes
P26	More arteries, stents or bifurcations than performed	Yes	Yes
P27	Catheter clot removal billed as open neck surgery	Yes	Yes
P28	Amputation billed above the level performed	Yes	Yes
P29	Skin repair billed a layer too deep	Yes	Yes
P30	Special-product device code with no product in the record	Yes	Yes
CATEGORY 6 — DRG MECHANICS			
P31	FY2027 new-family gates	Yes	Yes — backtest mode
P32	Surgery unrelated to the reason for admission (981–983)	Yes	Yes
P33	DRG that did not exist on the discharge date	No	OCTOBER ONLY
P34	Provider outlier monitor (feeds every concept above)	n/a	Yes

Green = no chart needed (claim proves itself). Amber = waits for October.

Category 1 — Data & Date Integrity

Run these first. Every other concept either uses dates or assumes the codes on the claim belong to this stay. If these return volume, fix the data build before sizing anything else.

P01 Procedure dated outside the hospital stay	
In simple words	A procedure on the claim is dated before the patient was admitted, or after they went home.
Why it's wrong	A hospital bills for what it did during this stay. Anything outside the admission window does not belong on this bill. In your file some procedure dates sit in 2022 and 2026 next to 2025 admissions, which suggests dates from other visits are leaking in.
Guideline	UHDDS: procedures are reported for the admission in which they were performed. Federal Register 31-Jul-1985: the record must substantiate what drove the DRG.
What it's worth	Usually a data-quality finding rather than a payment finding — but it decides whether you can trust everything else.
Run status	Runs today. No chart needed.

P02 Procedure dates impossibly far apart on one claim	
In simple words	The earliest and latest procedure dates on one claim are much further apart than the length of stay.
Why it's wrong	A four-day admission cannot contain procedures three months apart.
Guideline	Same as P01. UHDDS admission-scope rule.
What it's worth	Data integrity. Confirms or rules out the leakage P01 suggests.
Run status	Runs today. No chart needed.

P03 Same code, same date, billed twice	
In simple words	The identical procedure code appears more than once with the same date on one claim.
Why it's wrong	One thing that happened once should be billed once.
Guideline	PCS B3.2: multiple procedures are coded when they are genuinely distinct performances.
What it's worth	Modest on its own — but critical to settle, because if these are an artifact of how your file is built, then every concept that counts occurrences (fusion levels, PCI arteries, bilateral pairs) is being fed inflated numbers.
Run status	Runs today. IMPORTANT: the same code on DIFFERENT dates is legitimate — a dialysis patient treated Monday, Wednesday and Friday will correctly have three identical codes. Those are excluded.

Category 2 — Impossible On Its Face

The claim disproves itself. Highest confidence, lowest cost, hardest to appeal. This is where a new programme should start.

P04 Over 96 hours of ventilation in a stay shorter than that	
In simple words	The claim bills the 'more than 96 hours on a ventilator' code, but the whole stay was four days or less.
Why it's wrong	Ninety-six hours is four full days. The ventilator time cannot exceed the entire admission.
Guideline	Coding Clinic 4Q 2014 pp.3–4 (count from intubation to extubation, weaning included; if the patient arrived ventilated, count from admission). PCS A8.
What it's worth	The >96-hour code is one of the widest single-code payment splits in the system — it separates DRG 207 from 208 and is a required element of the top-paying DRGs 003/004 and of the burn families.
Run status	Runs today. No chart needed to select; the vent flowsheet confirms.

P05 Two conflicting ventilation duration codes	
In simple words	Both the 'over 96 hours' code and the '24 to 96 hours' code on one claim.
Why it's wrong	One continuous episode on a ventilator has one duration. It cannot be both.
Guideline	PCS B3.2 (one continuous performance = one code); Coding Clinic 4Q 2014.
What it's worth	Either it is a duplicate, or two genuinely separate episodes occurred — in which case the record must show two clear start and stop times. Otherwise the longer code comes off.
Run status	Runs today. Your own data contains examples.

P06 Two separate 'over 96 hour' ventilation episodes	
In simple words	The >96-hour code appearing on two different dates within one admission.
Why it's wrong	This means the patient was ventilated for over four straight days, came off, then went back on for over four more. Possible — but rare enough to verify every time.
Guideline	Coding Clinic 4Q 2014; PCS B3.2; UHDDS substantiation.
What it's worth	Each episode must have documented start and stop times. Your data shows a claim with this code on 30 January and again on 23 February.
Run status	Runs today.

P07 Top-paying DRG 003/004 with no tracheostomy code	
In simple words	A claim grouped to DRG 003 or 004 with no tracheostomy code (0B11) anywhere on it.
Why it's wrong	Those two DRGs are among the highest-paying in the entire system and specifically require a tracheostomy — a surgical opening in the neck. No tracheostomy code, no qualification.
Guideline	MS-DRG Definitions Manual Pre-MDC logic; PCS root operation Bypass (0B11 = bypass trachea to cutaneous); PCS A8; UHDDS substantiation.
What it's worth	The single largest per-claim recovery in this library. The claim falls out of the Pre-MDC pair to a normal respiratory DRG.
Run status	Runs today. No chart needed to select.

P08 A throat tube or tube swap treated as a tracheostomy	
In simple words	DRG 003/004 claims where the only airway procedure is a breathing tube down the throat (0BH1, e.g. 0BH17EZ) or a change of an existing tube (0B21).
Why it's wrong	A tube down the throat is intubation — temporary, no surgery. Changing an existing tube is just a swap of a device already there. Neither one creates a tracheostomy. The three code families look similar at a glance, which is exactly why this is missed.
Guideline	PCS root operation definitions: Bypass (0B11, creates the tracheostomy) vs Change (0B2, swaps an existing device) vs Insertion (0BH, places an airway). PCS A11: no assumptions.
What it's worth	Same as P07 — the claim exits the top-paying pair. This is the specific trap worth building into every trach check.
Run status	Runs today. Your data contains claims with only 0BH17EZ and no 0B11.

Category 3 — Procedure Contradicts the Diagnosis

The two halves of the claim disagree. Cheap to select; most need a quick look at the note to confirm.

P09 Fracture 'put back in place' that was never displaced	
In simple words	The reposition code billed while the diagnosis on the same claim says the fracture was NONDISPLACED.
Why it's wrong	Reposition means moving a bone back where it belongs. If the bone never moved out of place, there is nothing to reposition — pinning it is a different, lesser code (Insertion of a fixation device).
Guideline	PCS root operation definition of Reposition; PCS A11; Coding Clinic on fixation of nondisplaced fractures.
What it's worth	Reposition-with-fixation is the surgery code driving the hip, femur and extremity surgical DRGs. Correcting it steps the claim down or collapses it to medical.
Run status	Runs today. Pure claim-versus-claim contradiction — no chart needed to select.

P10 Deep debridement with no deep-infection diagnosis	
In simple words	Muscle or bone debridement billed, with no matching deep-infection diagnosis anywhere on the claim.
Why it's wrong	Debridement is coded to the deepest layer the surgeon actually wrote down — not the deepest layer that seems plausible. If a surgeon really cut into muscle or bone, a supporting diagnosis is almost always present. Its absence suggests the depth was coded deeper than documented.
Guideline	PCS B3.5 (overlapping layers — code the deepest layer documented); PCS A8; AHIMA compliant query practice (2022) where the depth came from a query.
What it's worth	Each layer deeper buys a higher surgical tier. Corrected to the documented layer the DRG steps down; if it was the only surgery code, the claim goes medical.
Run status	Runs today. Confirm in the note: does it say muscle, fascia, or bone?

P11 Spine bone surgery with a disc diagnosis (or the reverse)	
In simple words	A procedure coded to the vertebral bones while the diagnosis is a disc problem — or a disc procedure alongside a bone diagnosis.
Why it's wrong	The bones and the discs are different structures, in different chapters of the coding book, routing to different DRG families.
Guideline	PCS B4.1a (code the body part actually treated); vertebral column body-part keys; PCS A6.
What it's worth	The wrong chapter can claim both the wrong surgical family and a surgical status the correct code would not carry.
Run status	Runs today. VERIFY the vertebral body-part values against the tables before relying on level-specific precision.

P12 Brain artery coded when the neck artery was treated	
In simple words	A clot removal or dilation coded to the intracranial artery, on a claim whose diagnosis supports only neck-vessel (extracranial) disease.
Why it's wrong	Intracranial procedures sit near the craniotomy DRG families — a much richer neighbourhood than the carotid families.
Guideline	PCS B4.1a; upper-artery body part key; PCS A11.
What it's worth	Wrong body part claims the intracranial premium.
Run status	Runs today.

P13 Spinal cord release coded when a nerve root was freed	
In simple words	Decompression billed as release of the spinal cord, on a claim whose diagnosis shows nerve-root symptoms only and no cord involvement.
Why it's wrong	The cord and the peripheral nerve roots are different chapters routing to different families.
Guideline	PCS B4.1a; central vs peripheral nervous system body-part keys; PCS A11.
What it's worth	The cord chapter routes to higher spinal families.
Run status	Runs today. VERIFY the cord body-part values; the diagnosis legs carry most of the precision.

P14 Patient's own skin graft with no donor site	
In simple words	A skin graft coded as using the patient's own skin, with no harvest code anywhere on the claim.
Why it's wrong	Skin taken from the patient has to come from somewhere, and the harvest is coded separately. No harvest usually means the truth is donor skin, artificial skin, or a temporary covering — all coded differently and often grouped differently.
Guideline	PCS B3.9 (autograft harvested from a different site is coded separately); device key (autologous vs synthetic vs nonautologous); PCS B6.1.
What it's worth	Sits inside the wound/graft and burn DRG families, where the graft code is a payment gate.
Run status	Runs today. A self-contradicting claim — no chart needed to select.

Category 4 — Coded Twice / Unbundled

One event billed as two, or a step billed separately from the operation it was part of. All selectable from code combinations alone.

P15 Bleeding control plus the operation that stopped it	
In simple words	A 'control of bleeding' code alongside a definitive operation on the same claim.
Why it's wrong	If the surgeon ended up doing something definitive to stop the bleeding, that definitive operation is what gets coded — the control attempt is included in it.
Guideline	PCS B3.7 (if control fails and a more definitive root operation is performed, code only the definitive one).
What it's worth	Control is a surgery code. On a medical bleeding admission it can be the code that wrongly makes the whole claim surgical.
Run status	Runs today. IMPORTANT: the two codes sit in different sections — control in body regions (0W3/0X3/0Y3), the operation in the organ system. Do not search for them under the same body system or you will find nothing.

P16 A 'look inside' billed with the surgery it was part of	
In simple words	An inspection code next to another procedure performed through the same route, same body system.
Why it's wrong	Looking at the organ was how the surgeon reached what they were operating on — not a separate service.
Guideline	PCS B3.11a and B3.11b.
What it's worth	Usually cleans padded procedure lists; matters most when another finding removes the main code and the leftover inspection must not keep the claim surgical.
Run status	Runs today.

P17 Keyhole and open version of the same operation	
In simple words	Both the laparoscopic and the open version of one operation, same claim.
Why it's wrong	The surgeon started keyhole and converted to open. That is one operation. Correct coding is the open procedure plus an inspection for the attempted approach.
Guideline	PCS B3.2d (procedure converted from one approach to another).
What it's worth	Inflates the procedure list and, where approach decides the DRG split, claims the wrong family.
Run status	Runs today.

P18 Aborted operation billed as if completed	
In simple words	A full definitive operation billed on a claim that also carries an inspection of the same body system — the signature of a case that was started and abandoned.
Why it's wrong	When an operation is aborted, only what was actually done gets coded — usually just the look inside.
Guideline	PCS B3.3 (discontinued or incomplete procedures); PCS A11.
What it's worth	The definitive code is a surgery code; inspection usually is not. Correcting it moves the claim from surgical to medical.
Run status	Runs today. The note confirms with words like aborted, unsuccessful, unable to cross, converted to diagnostic.

P19 Whole-organ removal and partial removal together	
In simple words	Both a resection (all of a body part) and an excision (part of it) on the same body system, same claim.
Why it's wrong	In this coding system a lung lobe or liver lobe counts as its own body part. Taking a wedge is excision; taking the whole lobe is resection. Billing both for one operation overstates what happened.
Guideline	PCS B3.8 (Excision vs Resection with body-part subdivisions); PCS B4.1a.
What it's worth	Resection anchors the major-procedure tier of its organ family; correcting it steps the claim down a tier.
Run status	Runs today.

P20 Spinal cage plus a separate bone-graft code, same level	
In simple words	Two fusion codes for the same spinal level — one showing a cage, one showing bone graft.
Why it's wrong	When a cage is used at a level, the cage is what gets coded. Adding a second code for the graft packed around it is billing one fusion twice.
Guideline	PCS B3.10b (device value hierarchy: interbody device outranks autologous, which outranks nonautologous tissue); B3.10c.
What it's worth	Fusion DRGs split on levels and complexity, with large weight gaps — including the new high-weight FY2027 fusion families.
Run status	Runs today.

P21 Joint liner swap billed as a full revision	
In simple words	A removal code and a replacement code for the same joint on the same day.
Why it's wrong	Replacement means putting in a device that takes the place of the body part. If the surgeon only swapped a plastic liner and no component was taken out, that is not a replacement.
Guideline	PCS root operation definitions (Replacement, Removal, Revision); Coding Clinic 4Q 2016 on liner exchange; PCS A11.
What it's worth	Historically claimed the higher revision DRGs. Note that from October 2026 the grouper sends this pair to a different family — so the finding stays valid but its destination changes.
Run status	Runs today. The note confirms whether a component was actually explanted.

Category 5 — The Note Must Be Read

Claims data tells you which claims to pull; a reviewer confirms in the record. Higher value per claim, higher review cost.

P22 Wound 'cut out' when the note says only washed	
In simple words	Excisional debridement billed — a surgery code meaning dead tissue was cut out with a blade — where the note describes only washing, irrigating, scrubbing or scraping.
Why it's wrong	Cutting tissue out and scraping tissue away are two different root operations with two different codes. The note has to describe cutting.
Guideline	PCS definitions of Excision vs Extraction; PCS A11 (map documentation to the definition, never assume); PCS A8; Coding Clinic 1Q 2017 (documentation must describe excisional debridement).
What it's worth	The highest-volume finding in your own audit history. On an ulcer or burn admission this is often the only surgery code, so removing it collapses the claim to medical. Its destination DRG family also received the largest weight increase in the FY2027 table.
Run status	Runs today. Look in the note for: excised, cut away, sharp excision, blade — versus irrigated, curetted, lavage, scrubbed.

P23 Airway biopsy or lavage billed as lung surgery	
In simple words	A diagnostic excision of a lung lobe through a scope, on a claim whose picture is a routine pneumonia or cancer workup.
Why it's wrong	The bronchoscopy report often shows a biopsy of the airway wall or a lymph node — a different body part, and not a surgery code — or only a washing, which is drainage, not excision at all.
Guideline	PCS B4.1a (code the body part actually sampled); Excision vs Drainage definitions; Coding Clinic 3Q 2016 on transbronchial vs endobronchial biopsy; PCS A8.
What it's worth	The lobe-excision code pulls a medical respiratory admission into the surgical chest DRGs. Corrected either way, the claim returns to its medical DRG. High volume with a full surgical-to-medical swing.
Run status	Runs today.

P24 Surgical claim with no operative report	
In simple words	A claim that is surgical only because of one operating-room procedure — check whether an operative report for it exists in this stay, on the matching date.
Why it's wrong	A hospital cannot be paid for surgery it cannot show. The record must prove what drove the payment, and a report from a different encounter does not count.
Guideline	Federal Register 31-Jul-1985 and UHDDS (the record must substantiate the DRG); PCS A8.
What it's worth	The biggest single-claim swing available: the entire surgical payment falls away and the claim regroups to the medical DRG for the diagnosis.
Run status	Runs today for selection; the report check happens once the record arrives.

P25 Single-level fusion billed as multi-level	
In simple words	A fusion code whose body part says 'two or more joints' where the note describes one level.
Why it's wrong	Each vertebral joint is its own body part in this coding system, so the number of levels is a coded fact that has to match the operation.
Guideline	PCS B3.10a (each vertebral joint is a separate body part).
What it's worth	Fusion DRGs split on levels and complexity with large weight gaps — and FY2027 added new high-weight fusion families, making an unearned level count worth substantially more from October.
Run status	Runs today. Careful: only certain body-part values mean 'two or more' — several similar-looking values are single joints and must not be flagged.

P26 More arteries, stents or bifurcations than performed	
In simple words	A cardiac stent claim whose code says multiple arteries, or a drug-eluting stent, or a bifurcation.
Why it's wrong	The code counts arteries treated, not stents placed — three stents in one artery is still one artery. The device character must match the stent actually deployed, and a bifurcation qualifier needs bifurcation treatment documented.
Guideline	PCS B4.4 (coronary arteries classified by number treated); device character definitions; Coding Clinic 2Q and 4Q 2016.
What it's worth	Moves the stent DRG splits on a very high-volume service line.
Run status	Runs today. Confirm against the catheterisation report.

P27 Catheter clot removal billed as open neck surgery	
In simple words	A carotid artery clot-removal code with an open approach — which describes a full surgical operation on the neck.
Why it's wrong	If the record shows a catheter procedure through a needle puncture, the approach character is wrong, and the approach alone decides which surgical family pays.
Guideline	PCS approach definitions (Open vs Percutaneous); PCS B5.3; PCS A11.
What it's worth	Claims the endarterectomy-level payment for a catheter procedure.
Run status	Runs today.

P28 Amputation billed above the level performed	
In simple words	An amputation code whose level is higher than the operation described — a below-knee amputation coded at the thigh, or a partial-foot amputation coded as the whole foot.
Why it's wrong	Amputation DRGs split by site and level; the higher level claims the heavier group.
Guideline	PCS Detachment body-part and qualifier definitions; PCS A11; PCS A8.
What it's worth	One level higher buys a heavier DRG.
Run status	Runs today. VERIFY: I am not confident enough in the amputation body-part values to encode a level test, so this concept is deliberately written to pull all amputation claims and reconcile the level at review. Lower precision, but no risk of a wrong-level false finding.

P29 Skin repair billed a layer too deep	
In simple words	Emergency-department laceration repairs and small excisions coded to the tissue under the skin with an open approach.
Why it's wrong	If the note supports a repair of the skin itself, done externally, the correct code is a different chapter and a different approach — and it is not a surgery code, while the deeper version often is.
Guideline	PCS B4.1a; PCS B5.2 (work performed directly on the skin is an external approach); PCS A11.
What it's worth	On a simple wound admission this is often the only surgery code, so the correction collapses the claim to medical. It was also the largest single swap cluster in your own audit data.
Run status	Runs today.

P30 Special-product device code with no product in the record	
In simple words	A new-technology device code — the kind that names one specific manufactured product.
Why it's wrong	These codes exist for one product each. If the implant log, invoice or pharmacy line shows a generic cage or standard bone cement instead, the special code does not belong.
Guideline	PCS Section X definitions (product-specific by design); PCS B6.1; new-technology payment substantiation.
What it's worth	Pure payment elevators: from October 2026 one of these codes can substitute for a major complication in reaching a higher DRG. Cheap to verify against an invoice, and hard to argue with.
Run status	Runs today for the products already in use. VERIFY the exact code list against the FY2027 final rule tables before treating the new DRG behaviour as live.

Category 6 — DRG Mechanics

P31 FY2027 new-family gates	
In simple words	The new October 2026 DRG families — extensive spinal fusion, single joint revision, knee and hip infection, consolidated pacemaker, rebuilt uterine cancer surgery — each have entry requirements that can be miscoded.
Why it's wrong	A brand-new DRG family has no coding history behind it, so first-year error rates are at their highest. Each family has a gate: a level count, a device, an infection diagnosis, a procedure type.
Guideline	CMS FY2027 IPPS final rule (family creation and grouper logic); plus the underlying PCS rules for each gate (B3.10 for fusion, root operation definitions for revision, B4.1a for anatomy).
What it's worth	The fusion families carry the largest weight cliff in the grouper, so an unearned gate element is worth a great deal.
Run status	Runs today in BACKTEST mode — the populations already exist under the predecessor DRGs the new families were carved from, so hit rates can be measured now. Switch to the new DRG numbers in October; only the payment weights change.

P32 Surgery unrelated to the reason for admission	
In simple words	Claims grouped to the 'unrelated operating room procedure' DRGs.
Why it's wrong	These DRGs literally mean the surgery has nothing to do with why the patient came in. Usually that is a coding mistake — the procedure was coded to the wrong body system, or the wrong condition was named as the main reason for admission.
Guideline	MS-DRG Definitions Manual unrelated-OR logic; UHDDS principal diagnosis definition; PCS B4.1a.
What it's worth	Few claims, high dollars each, near-100% review value. Correcting either half regroups the claim to a normal, cheaper family.
Run status	Runs today. Pull every one.

P33 DRG that did not exist on the discharge date	
In simple words	A discharge on or after 1 October 2026 billed with one of the eighteen DRGs deleted that day — or an earlier discharge billed with one of the new ones.
Why it's wrong	The payment was calculated from a price list that does not exist for that date.
Guideline	CMS FY2027 IPPS final rule effective dates; MS-DRG grouper version control.
What it's worth	Zero judgment, every hit a recovery. It also catches lagging grouper updates at clearinghouses in the first weeks of the new year.
Run status	OCTOBER ONLY — this is the one concept that cannot run today, because the date IS the logic. In the meantime, run the readiness query in the SQL file: it counts how many of your current claims sit in the DRGs about to be deleted, which tells you how much October exposure to plan for.

P34 Provider outlier monitor	
In simple words	Not a finding — a ranking. Compares each facility against its peers on the rates that matter: debridement coding, ventilation over 96 hours, single-code severity, surgical share.
Why it's wrong	Coding patterns cluster by facility. A hospital far outside its peer range is where your review budget produces the most hits per chart pulled.
Guideline	This is analytics, not a coding rule. Adjust for case mix before drawing any conclusion, and use the output only to prioritise review — never as a finding in itself.
What it's worth	Multiplies the yield of every other concept by pointing them at the right claims first.
Run status	Runs today.

Smaller concepts worth adding once the core is running

These are real and defensible, but low volume — build them after the thirty-four above are live. **Vessel embolization intent:** partially narrowing a vessel and completely closing it are different root operations, and the angiogram report states which was intended (PCS root operation definitions). **Temporary heart pump qualifier:** a percutaneous heart-assist device placed and removed within the stay needs the short-term qualifier, or the claim is exposed to the implant family (PCS qualifier definitions; B6.1). **Stem cell transplant source:** one character separates the patient's own cells from a donor's, and the donor version groups to a much higher-paying family — so donor-coded claims must show donor documentation (PCS qualifier definitions; A8). **Lymph node sampling ladder:** removing a whole node chain, removing one node, and needle sampling are three different codes, and the top rungs are surgery codes (PCS B3.8).

Crosswalk from the earlier documents

Nothing has been lost. If you already started work under an old ID, find it here.

New ID	Was previously
P01–P03	Red-flag checks A1/A2/A3, B1
P04–P06	C11 (vent legs), C50 legs L1/L2/L3
P07–P08	C11 Pre-MDC guard, C50 legs L4/L5
P09	C27
P10	C02
P11	C16
P12	C20
P13	C22
P14	C47
P15	C13
P16	C14
P17	C05
P18	C04
P19	C06
P20	C08b
P21	C09 / C33 / C34
P22	C01 and C46 (burn scope)
P23	C07 and C19 (lavage variant)
P24	C03 and C45 (antepartum)
P25	C08a
P26	C10
P27	C18
P28	C12
P29	C17
P30	C30 and C31
P31	C29, C32, C36, C38, C39, C42, C44 (all FY2027 gates, consolidated)
P32	C28
P33	C35
P34	F04 / C41 monitors
Appendix	C25, C23, C43, C21
Retired	C24 (diagnostic/therapeutic qualifier flip) and C15 (drainage device by length of stay) — both too imprecise to be worth review time. C37 and C40 were revaluation sweeps rather than distinct errors; their economics are noted inside P22 and P31.

Where to start, in order

Week 1: P01–P03. Settle whether your dates and duplicates are trustworthy. Nothing else is reliable until they are. **Week 2:** P07 and P08 — the tracheostomy checks. Biggest dollars per claim, almost no judgment, and your data already shows candidates. **Week 3:** P04, P05, P09, P14 — the remaining self-contradictions, all selectable with no chart. **Week 4**

onward: P22 and P24, the two highest-volume chart-based concepts, using P34 to point them at the right facilities first. **Backtest P31** in parallel so you know your October exposure before October arrives. **Park P33** until then.

Two honest cautions

First, three concepts are marked VERIFY (P11, P13, P28) because I am not fully confident of specific body-part values from memory. Those are written wide on purpose — they will pull more claims than strictly necessary rather than risk a wrong-level finding. Have a coder confirm the values against the current code tables once, and they can then be tightened. Second, the payment amounts behind FY2027 concepts stay provisional until CMS releases the pricing file, and every recovery calculation must regroup the claim on the grouper version matching its discharge date — not the current one.